



Pharm*Achieve*

INTRA- AND INTER- PROFESSIONAL COLLABORATION

Qualifying Exam Preparatory Course

COMPETENCIES COVERED IN THIS PRESENTATION...

Providing Care:
Clinical Care

Providing Care:
Distribution

Knowledge
& Expertise

Communication
& Collaboration

Leadership &
Stewardship

Professionalism

LEGEND

ACP Advanced Care Paramedic	ISMP Institute for Safe Medication Practices
APN Advanced Practice Nurse	IV Intravenous
ASA Acetylsalicylic Acid	MSK Musculoskeletal
BID Bis In Die (Twice Daily)	NP Nurse Practitioner
CBT Cognitive Behavioural Therapy	NSTEMI Non-ST-Elevation Myocardial Infarction
CCP Critical Care Paramedic	OT Occupational Therapists
CKD Chronic Kidney Disease	PCP Primary Care Paramedic
COPD Chronic Obstructive Pulmonary Disease	PO Per Os (Oral)
COVID Coronavirus Disease	PRN Pro Re Nata (As Needed)
DTP Drug Therapy Problem	PT Physiotherapists
DUE Drug Utilization Evaluation	Q Quaque (Every)
eGFR Estimated Glomerular Filtration Rate	R&D Research And Development
EN Enteral Nutrition	RDL Remote Dispensing Location
FPG Fasting Plasma Glucose	RN Registered Nurse
GERD Gastroesophageal Reflux Disease	RPN Registered Practical Nurses
HbA1c Glycated Hemoglobin	RSV Respiratory Syncytial Virus
HCP Health Care Provider	SL Sublingual
HTN Hypertension	TDM Therapeutic Drug Monitoring
IR Immediate-Release	TPN Total Parenteral Nutrition

TABLE OF CONTENTS

Role of Pharmacist in

- Community practice
- Hospital practice
- Other healthcare settings
- Industry

Role of other healthcare providers

- Nurses, dietitians, optometrists, etc.

Delegation and Collaboration

LEARNING OBJECTIVES

- 1 Discuss the role of the pharmacist in improving patient outcomes, healthcare costs and medication use
- 2 Differentiate between the role of community pharmacists and hospital pharmacists
- 3 Understand pharmacists' responsibilities within the healthcare system as it relates to optimal medication management

INTRA- VS INTER- PROFESSIONAL COLLABORATION

Inter-professional Collaboration

- Partnership between healthcare professionals with different backgrounds and a client
- Requires:
 - Role clarification
 - Conflict resolution
 - Team functioning
 - Collaborative leadership
- Purpose: shared decision-making around health and social issues
- Example: team of physicians, nurses, pharmacists, and specialist working together

Intra-professional Collaboration

- Partnership between healthcare professionals with the same background and a client
- Example: team of pharmacists and pharmacy technicians working together

PHARMACIST'S ROLE: COMMUNITY PRACTICE

- “Classic” dispensing model
 - Compounding
 - Dispensing
 - Limited patient/healthcare interaction
- Drug product line expansion, costs and volume increase
 - As complexity increases, the demand for specialized knowledge grows
- Pharmacy mandate to counsel and to be the drug therapy expert within a pharmaceutical care paradigm
- Increased autonomy with increasing scope of practice

Technical Duties:

- Prescription processing
 - Order entry
 - Adjudication
 - Filling
 - Checking
 - Purchasing and Receiving

Clinical Duties:

- Counseling
- Patient follow-up
- Student mentorship
- Continued on the next slide →

PHARMACIST'S ROLE: COMMUNITY PRACTICE

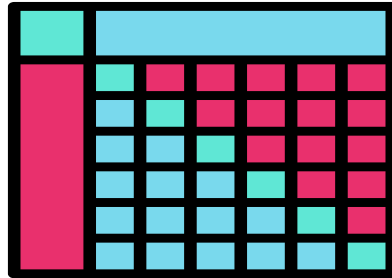
Expanded Scope of Practice

- Provide immunizations (additional training and certification required)
- Extend prescriptions for continuing care
- Adapt dose, formulation, regimen, duration and route
- Therapeutic substitution
- Optimize drug therapy for chronic conditions (diabetes, HTN)
- Run clinics e.g. flu, diabetes, HTN clinics...etc.
- Prescribing in emergency situations
- Minor ailments prescribing (jurisdictions vary by region)
- Point-of-care testing (A1c, strep throat)
- Improve patient outcomes (reduce adverse drug reactions, opioid harms reduction)
- Decrease cost to health care system (hospital emergency admissions)
- Order blood work to assess patient and make recommendations
- Prescribing under delegation, prescribing within a collaborative agreement/relationship



On the OSCE and MCQ, they will accept as true any answer that is true of the role of the pharmacist in **any** Canadian jurisdiction

PHARMACIST'S ROLE: COMMUNITY PRACTICE



"Scope" of practice define the services that members of an occupation perform and the methods they use



Canadian Pharmacists Association | Scope of Practice

PHARMACIST'S ROLE: HOSPITAL PRACTICE

Clinical Pharmacists

Drug Information Pharmacist

Anti-Microbial Stewardship

Pharmacy Manager(s) and Director

PHARMACIST'S ROLE: HOSPITAL PRACTICE

Patient-Centered Practice

- Optimize medication therapies and make recommendations based on diagnosis, patient history and clinical factors
- Monitor efficacy and safety of therapy
- Notify other members of the team of potential risks associated with drug therapy
- Perform clinical pharmacokinetic and therapeutic drug monitoring (TDM)
- Medication reconciliation (on admission, transfer and discharge)
- Disease state management for chronic conditions (HTN, diabetes)

Drug Information Provider

- Provide up-to-date drug information

Perform Drug Utilization Evaluation (DUE)

- To ensure proper and optimal use of drug therapy for conditions
- To reduce cost by performing audits on high-cost drugs

What is an example of a drug that requires TDM?

PHARMACIST'S ROLE: HOSPITAL PRACTICE

Committee Member

- Pharmacy & Therapeutics
- Patient Safety

Clinical Specialist-Stewardship Pharmacists

- Antimicrobial stewards for infectious diseases

Interprofessional Collaboration

- Collaborate with other professionals to expand services, improve care and delivery
 - Medical directives to initiate therapies
 - Smoking cessation
 - Warfarin ordering
 - Pre-printed order development

PHARMACIST'S ROLE: HOSPITAL PRACTICE

Education and Mentorship

- Educate patients on **optimal medication use**
 - Improve adherence and prevent re-hospitalization
 - **Discharge medication counselling**
- Educate staff on medications → Physicians, nurses, other allied health professionals
- Participate in student training; clinical rotations

Patient Safety Initiatives

- Policy and procedure development
- **Improve safety through internal process review** of pharmacy distribution delivery systems and medication management

Technical Duties

- **Prescription processing** (order entry, filling, checking)

PHARMACIST'S ROLE: AMBULATORY SETTING

- Medication management experts
- Manage chronic conditions (HTN, diabetes, pain) in clinic settings
- Home care services for elderly
- Long-term care patients

PHARMACIST'S ROLE: REMOTE DISPENSING

- A pharmacist does not have to be physically present in the remote dispensing location (RDL) but they need to be physically present in the pharmacy that operates the RDL
- Oversee the dispensing process through audiovisual equipment
- Pharmacists are still **required to counsel patients and verify appropriateness** of drug before it is released
- Advantage in remote areas where pharmacies are limited
- Requires a **certificate of accreditation** from the College of Pharmacy
- Needs to be operated in accordance with the regulations

PHARMACIST'S ROLE: ACADEMIC DETAILING

- An educational approach where one healthcare provider (e.g. pharmacist) interacts face-to-face with another healthcare professional to provide evidence-based information regarding a specific health topic of interest, with the purpose of optimizing patient care
- Provide rationale, cost-effective, evidence-based medicine
- Use clinical insight and experience
- Optimize the use of medications using the above structure
- **Objective, Free of pharmaceutical bias**
- Credible source of information
- Use newsletters, prescribing aids, patient information, personalized one-on-one education, risk calculators, continuing education programs
 - RxFiles®, Provincial Drug Utilization programs or academic detailing services
 - E.g.  Use of Antipsychotics in Behavioural and Psychological Symptoms of Dementia
- Decreases cost through effective prescribing and evidence-based medicine

PHARMACIST'S ROLE: INDUSTRY

1. Research and Development (R&D)
 - Clinical trials
 - Progenitor drug development
2. Quality Control
3. Drug Information
4. Drug Reimbursement/Coverage
5. Legislation
6. Drug Safety
7. Pharmaceutical Sales
8. Executive/Management

PHARMACIST'S ROLE: ACADEMIA AND GOVERNMENT

1. Policy Development

- Promoted in all areas of the profession

2. Teaching

3. Research

4. Public Health

- Vaccinations
- Public awareness
- Emergency stockpiles

5. Formulary Control and Advisement

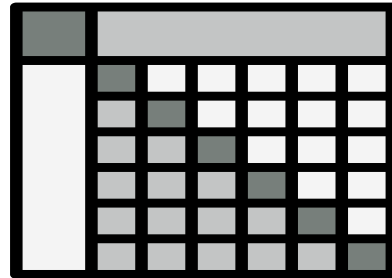
OTHER HEALTHCARE PROVIDERS

- Nurses
- Physicians/Physician Assistant
- Chiropractors/Podiatrists
- Chiropractors
- Dentists
- Physiotherapists
- Occupational Therapists
- Speech/Language Pathologists
- Dieticians
- Social Workers
- Psychologists
- Respiratory therapists
- Paramedics
- Pharmacy technicians



Each profession tends to **specialize in certain areas**, although skills and roles vary across the country and often overlap

BECOMING A HCP IN CANADA

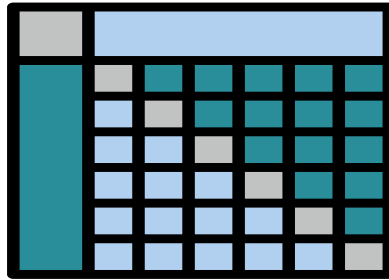


**Licensure/Certification
Requirements** of Selected Health
Care Providers (HCP) in Canada



Canadian Institute for Health Information
| Canada's Health Care Providers

BECOMING A HCP IN CANADA



Canadian Institute for Health Information
| Canada's Health Care Providers

NURSE'S ROLE

Place of Practice

- Hospital, community, long-term care/rehabilitation, public health, private industry, administration and government

Responsibilities

- Direct patient care; specific tasks dependent on knowledge, skill and judgement of each nurse along with practice site
- Administering medications, assisting in medical procedures, patient education
- Different classes
 - Registered Practical Nurses (RPN)
 - Registered nurses (RN)
 - Advanced Practice Nurse (APN)
 - Nurse Practitioners (NP)
 - RN with an expanded scope of practice
 - Authorized to diagnose, order and interpret lab tests, prescribe medications

PHYSICIAN'S ROLE

Place of Practice

- Hospital, community, long-term care/rehabilitation, public health, private industry, administration and government

Responsibilities

- Assessment of patients for diagnosis of diseases/condition and development of management/treatment strategy
- May practice as a general practitioner or have a specialization (e.g. anesthesiology, cardiology, neurology, pediatrics, psychiatry)
- May prescribe most drugs
- CanMEDS framework describes the abilities physicians need to meet health care needs of patients:
 - Leader
 - Communicator
 - Medical Expert
 - Health Advocate
 - Scholar
 - Professional
 - Collaborator

PHYSICIAN'S ROLE

Can all physicians prescribe **methadone** or **suboxone**?



DENTIST'S ROLE

Place of Practice

- Largely private practice or may be employed in hospitals, clinics, public health facilities or universities

Responsibilities

- Diagnose, treat, prevent and control disorders of the teeth and mouth
- Clean*, restore, extract and replace teeth, educate patients, perform oral surgery
- Have some prescribing privileges, including narcotics (e.g. oral antibiotics, analgesics, some narcotics)
- Dentists may specialize in areas such as oral and maxillofacial surgery, orthodontics, pediatric dentistry, periodontics, endodontics, prosthodontics, oral pathology, oral radiology or public health dentistry

*Although dentists can also clean teeth, this is typically done by dental hygienists

OPTOMETRIST'S ROLE

Place of Practice

- Largely private practice, clinics and community health centers

Responsibilities

- Examine eyes and prescribe eyeglasses and contact lenses
- Treat, manage and correct disorders and diseases of the visual system, the eye and its associated structures.
- Recognize and detect related systemic conditions and manage ocular manifestations.
- Have some prescribing privileges (e.g. ophthalmic drugs)

What is the difference between an optometrist and an optician?

CHIROPODIST/PODIATRIST

Place of Practice

- Largely private clinics, some hospitals, community health centers, family health teams, long term care or rehabilitation-based practice

Responsibilities

- Assess, diagnose, and treat medical conditions of the foot
- Chiropodist/Podiatrist:
 - Cut into subcutaneous tissue of the foot
 - Administer substances (e.g. injection into foot, inhalation)
 - Prescribe drugs designated in regulations
- Podiatrist can additionally communicate diagnosis that identifies a foot disease or disorder as the cause of a person's symptoms



Prescribing privileges, including narcotics* (e.g. antibiotics, antifungals)

*Cannot prescribe methadone, buprenorphine, anabolic steroids or designated drugs (e.g. amphetamine, methamphetamine)

CHIROPRACTOR'S ROLE

Place of Practice

- Largely outpatient/clinic, some hospital or rehabilitation-based practice

Responsibilities

- Diagnose and treat patients' neuromuscular-skeletal disorders of the spine and other body joints by adjusting the spinal column or through other corrective manipulation
- Management of various musculoskeletal (MSK) conditions including sprains/strains and neck and back pain
- Advice on corrective exercises, posture, lifestyle and nutrition

PHYSIOTHERAPIST'S (PT) ROLE

Place of Practice

- Outpatient and hospital or rehabilitation-based practice

Responsibilities

- Assess, treat, prevent and manage injury, pain, disease and/or disorders
- Optimize client's physical function, functional independence, mobility and quality of life
- Educate clients and others about health, wellness, fitness and self-management
- Treatment of acute injuries as well as management of chronic conditions such as diabetes, heart disease, pain

OCCUPATIONAL THERAPIST'S (OT) ROLE

Place of Practice

- Outpatient, hospital, rehabilitation-based practice, long-term care homes

Responsibilities

- Develop individual and group programs with people affected by illness, injury, developmental disorders, emotional or psychological problems and ageing to maintain, restore or increase their ability to care for themselves and to engage in work, school or leisure
- Develop and implement health promotion programs with individuals, community groups and employers
- OTs may specialize in a specific age group or disability such as those with arthritis, developmental coordination disorder, mental illness, or spinal cord injury

SPEECH/LANGUAGE PATHOLOGIST'S ROLE

Place of Practice

- Hospital or rehabilitation-based practice, public health centers, educational institutions, private practice

Responsibilities

- Identification, diagnosis, and treatment of human communication disorders including speech, language, voice and swallowing disorders
- May work with patients with a stroke, brain injury, hearing loss, developmental delay, Parkinson's disease, a cleft palate or autism

DIETICIAN'S ROLE

Place of Practice

- Hospital or rehabilitation-based practice, outpatient settings, public health

Responsibilities

- Planning, implementation and management of nutrition and food service programs directed at encouraging healthy nutrition outcomes and the prevention of nutritional disorders
- Dietitians provide treatment of nutrition-related diseases and conditions and work to ensure high-quality food service operations in a variety of settings
- May work with HTN, CKD, diabetic patients

SOCIAL WORKER'S ROLE

Place of Practice

- Hospitals, school boards, social service agencies, child welfare organizations, correctional facilities, employee assistance programs, private practice

Responsibilities

- Help individuals, couples, families, groups, communities and organizations develop the skills and resources they need to enhance social functioning and provide counselling, therapy and referral to other supportive social services
- Respond to other social needs and issues such as unemployment, racism and poverty
- Social workers may specialize in fields of practice such as child welfare, family services, corrections, gerontology or addictions

PSYCHOLOGIST'S ROLE

Place of Practice

- Hospitals, academic institutions, clinics, private offices, correctional facilities, employee assistance programs

Responsibilities

- Assessment and diagnosis of behavioural, emotional and cognitive disorders
- Counselling clients, providing psychotherapy (CBT, Mindfulness)
- Research and applying theory relating to behaviour and mental processes
- Work with patients to manage psychological factors that contribute to health and disease management
- Addresses patients' needs including mental health, stress, personal growth, home support, behavioural problems, violence, suicide, etc.
- **Do not have prescribing authority**

RESPIRATORY THERAPIST'S ROLE

Place of Practice

- Hospitals, patients' homes, rehabilitation centers, educational institutions, medical device manufacturers

Responsibilities

- Plan and implement therapeutic and diagnostic procedures for patients with cardio-respiratory disease (such as asthma, heart failure, COPD and drowning victims)
- Provide airway and breathing support in emergency rooms, intensive care units and operating rooms.
- Educate patients about lung health and use of breathing medications
- Administer a substance by injection or inhalation under orders from a physician

PARAMEDICS

Place of Practice

- Ambulances (first responders), assigned hospital emergency rooms, medical institution (education and research)

Responsibilities

- 3 levels of paramedics in Canada: primary care paramedics (PCP), advanced care paramedics (ACP) and critical care paramedics (CCP)
- PCPs provide basic medical care e.g. administration of meds (e.g. ASA, Epipen[®], nitroglycerin...etc.). They also perform controlled medical acts and procedures (e.g. defibrillation). ACPs can administer IV* medications as well as perform more controlled medical procedures (e.g. intubation). CCPs are focused mainly on critical and intensive care.

PHARMACY TECHNICIANS

Place of Practice

- Hospital, community and retail pharmacies, long-term care homes

Responsibilities

- Accountable and responsible for the technical aspects of both new and refill prescriptions:
 - Receiving and transcribing verbal prescriptions from practitioners (except narcotics, benzodiazepines and other targeted substances)
 - Ensuring that a prescription is complete and authentic
 - Transferring prescriptions to and receiving prescriptions from other pharmacies (except benzodiazepines and other targeted substances)
 - Ensuring the accuracy and performing final check of the drug preparation



Pharmacy technicians are now **regulated in 9 provinces**

PHARMACY TECHNICIAN'S CAN:

- ✓ Accept prescriptions from patients
- ✓ Accept verbal prescriptions (except narcotics, controlled drugs, benzodiazepines and targeted substances)
- ✓ Gather information and carry out order entry
- ✓ Select/prepare products
- ✓ Transfer prescriptions to another pharmacy on behalf of the pharmacist (except benzodiazepines and other targeted substances)
- ✓ Perform independent double check (verification for technical function)
- ✓ Provide information and educate patients (but cannot provide clinical or therapeutic judgment or recommendations)
 - For example, they can:
 - ✓ Read a product label to patient
 - ✓ Demonstrate or explain what the steps mean for an inhaler (based on how-to-use sheet)
- ✓ Accept delegation of a controlled act
- ✓ Administer certain vaccines (e.g. COVID, influenza, RSV) if injection certified (currently only in some provinces)

PHARMACY TECHNICIAN'S CANNOT:

- ✗ Answer questions that require therapeutic knowledge, clinical analysis or assessment (e.g. "Which product would you recommend for my cough?")
- ✗ Determine appropriateness of drug therapy
- ✗ Renew and adapt prescriptions
- ✗ Receive a verbal order for a narcotic or controlled drug
- ✗ Administer a drug*
- ✗ Witness ingestion of methadone or Suboxone®*

*May vary from one province to another

PRACTICE CASE

In the ICU what role would the following HCPs have?

Physician	
Pharmacist	
Dietician	
Respiratory Therapist	
Nurse	
Speech Language Pathologist	
Social Worker	

PRACTICE CASE

In the ICU what role would the following HCPs have?

Physician	• Diagnosis, prescriber, lead rounds
Pharmacist	• Identify DTPs, optimize drug therapy, TDM, antibiotic stewardship
Dietician	• Optimize feeds (EN or TPN) and micronutrients
Respiratory Therapist	• Optimize ventilators, provide guidance for sedation
Nurse	• 1:1 ratio with patients, follow labs, administer meds, vigilance of patients
Speech Language Pathologist	• Optimize speech post ventilator tube removal
Social Worker	• Help reduce family distress and provide relevant resources • Help with communication between patients, their families and the critical care team • Help answer questions on Power of Attorney

DELEGATION

Delegation is when an authorized healthcare professional that can perform a controlled act confers that authority to another person who is not authorized to perform the act

Regulated healthcare professionals are licensed to provide designated acts or activities, for example:

- “Controlled Acts” (Ontario)
- “Restricted Activities” (Alberta, British Columbia)
- “Reserved Acts” (British Columbia)

May only be performed by
authorized regulated
health professionals

Under appropriate circumstances, these acts or activities can also be delegated to others or performed under the supervision of someone licensed to undertake them

DELEGATION

Can take place through either a **direct order** or a **medical directive**

Direct Order

- Verbal or written order given as care is needed
- Initiates a specific intervention or treatment to be delivered at a specific time
- **One order** given for **one patient**

Medical Directive

- Written order developed ahead of time
- Pertains to any patient who meets criteria set out in the medical directive
- Can be **carried out multiple times** for **multiple patients**

Who Can Delegate?

- Pharmacist to technician
- Physician to pharmacist
- Physician to nurse
- Pharmacist to nurse

DELEGATION

Principles

Delegation of a controlled act should only occur if it is clinically appropriate and in the patient's best interests

The use of delegation is only appropriate to advance patient interests, not professional self-interest

Patient safety is paramount and a member should only accept delegation if he or she feels competent to perform the delegated act or procedure

A member is accountable for his or her performance and patient outcomes when accepting delegation



Document everything!

COLLABORATION



Collaborative care entails physicians and other providers using complimentary skills, knowledge and competencies, working together to provide care to a common group of patients based on trust, respect and an understanding of each others' skills and knowledge.

– *Canadian Medical Association*

Goals of Collaborative Care

- ✓ Improve access to care
- ✓ Enhance the quality and safety of care
- ✓ Enhance the coordination and efficiency of care
- ✓ Enhance provider morale and reduce burnout within health professions

COLLABORATION

Why is it important that pharmacists be involved in collaborative care?

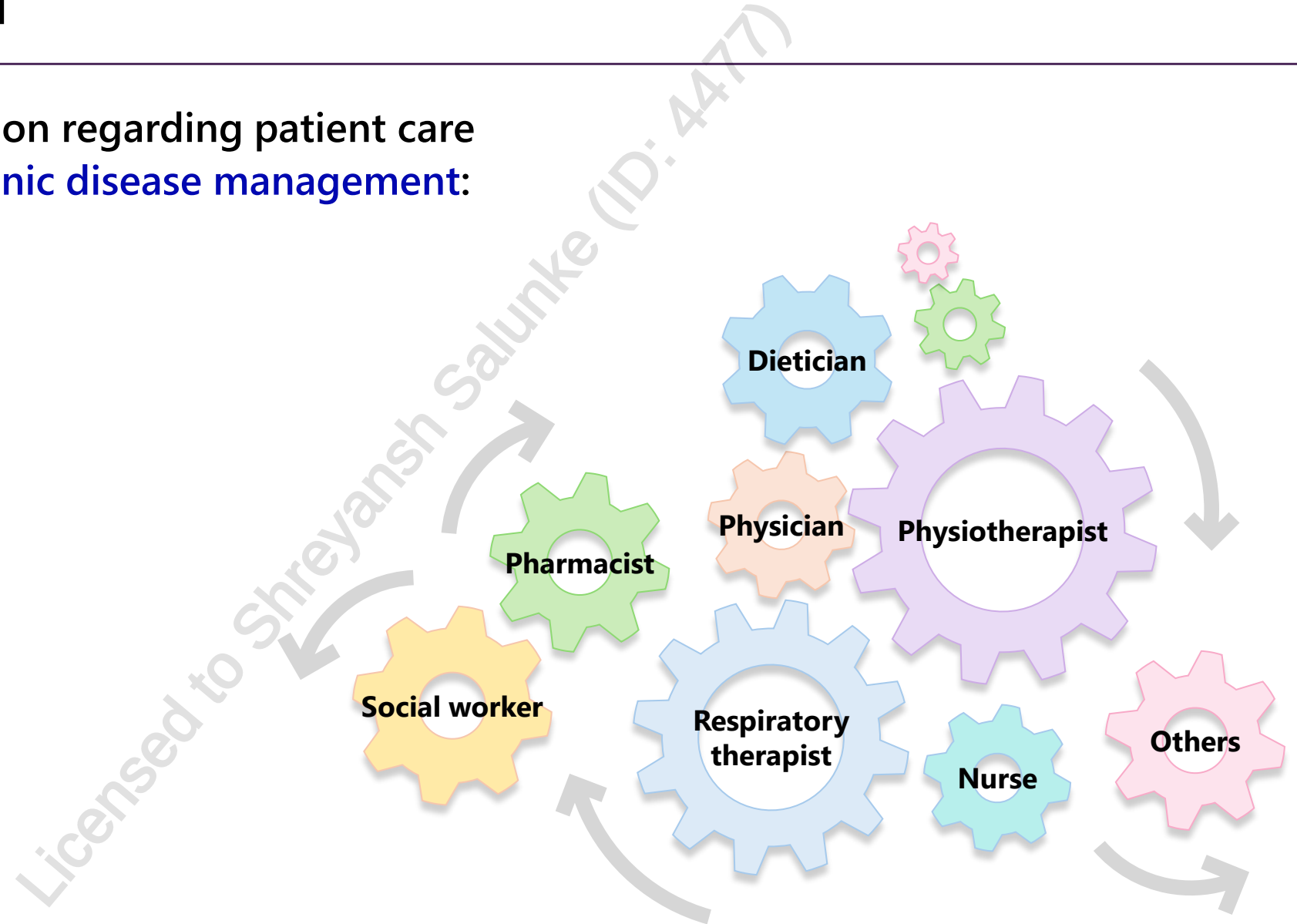
- Presence of drug related morbidity and mortality
- Rapid advancements and innovations in medicine and pharmaceuticals
- Growth of managed care (causing movement of patients from inpatient to ambulatory settings)
- Aging population with complex medical conditions requiring multiple providers



COLLABORATION

Interdisciplinary collaboration regarding patient care centers mainly around **chronic disease management**:

- Diabetes
- Hypertension
- Dyslipidemia
- Heart health
- Asthma/COPD
- Geriatrics
- Chronic pain
- Mental Health
- More..



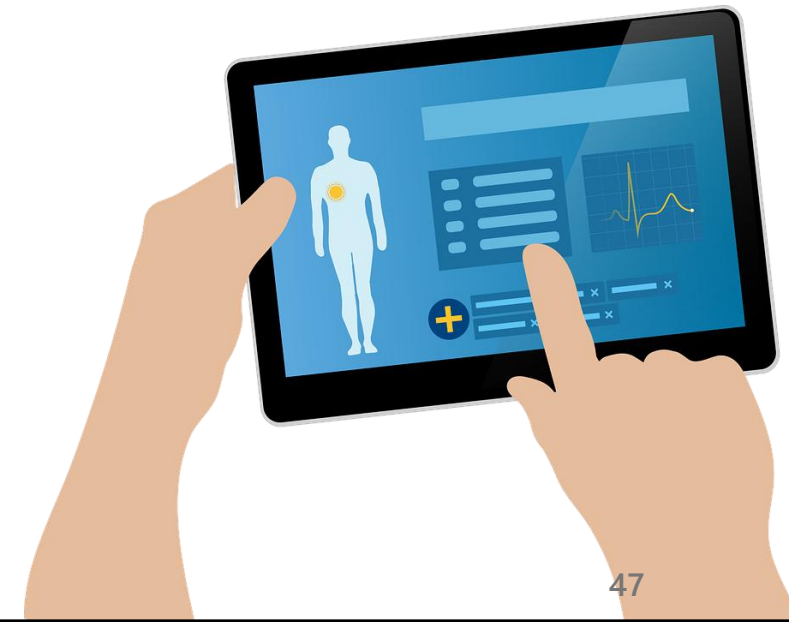
COLLABORATION

How to collaborate effectively:

- Understand and respect the roles and skills of providers within the team
- Listen to and value the input of team members in the care of a patient
- Communicate effectively with all members of the team

Future of collaborative care includes:

- Further delegation → more medical directives
- Need for improved communication mechanisms → electronic health records
- More professions allowed to perform more controlled acts



TAKEAWAY

Interprofessional collaboration is the partnership between healthcare professionals with *different* backgrounds to provide comprehensive patient care

Intra-professional collaboration is the partnership between healthcare professionals with the *same* background to provide comprehensive patient care

The role of a pharmacist may differ depending on their area of practice e.g. community, hospital, industry, ambulatory, remote dispensing, academic detailing, academia and government

Other professions tend to specialize in certain areas, although skills and roles vary across the country and often overlap

It is important to understand the role of other healthcare professionals to assist in collaborative patient care

Delegation can take place through a direct order or a medical directive

CASE STUDY

You are the pharmacist in a multi-disciplinary family health team clinic. JS is a 70-year-old male who presents to the clinic for an annual medication review. His past medical history is significant for type 2 diabetes mellitus, a non-ST-elevation myocardial infarction (NSTEMI) 8 years ago, and osteoarthritis of the knee. He routinely visits his physician, the dietitian, the physiotherapist, and the nurse practitioner at the clinic. His current medications include empagliflozin 5 mg PO BID, metformin 500 mg PO BID, rosuvastatin 40 mg PO daily, Viacoram® (perindopril arginine 14 mg/amlodipine 10 mg) PO daily, acetylsalicylic acid 81 mg PO daily, metoprolol tartrate immediate-release (IR) 100 mg PO BID, nitroglycerin 0.4 mg SL, and naproxen 500 mg PO BID. JS mentions that he struggles with taking as many pills as he does every day. He reports that his osteoarthritis knee pain is not well controlled with a pain score of 6/10 on most days of the week. He denies smoking or alcohol use. JS informs you that he exercises once a week on the treadmill for 45 minutes and he does not follow a strict diet. He weighs 162 lbs and is 160 cm tall.

Laboratory test results returned today, revealing: an estimated glomerular filtration rate (eGFR) of 90 mL/min/1.73m², a fasting plasma glucose (FPG) level of 5.0 mmol/L, and an HbA1c of 6.9%. His blood pressure was recorded this morning in the clinic as 133/81 mmHg, and he routinely records and measures his blood pressure at home every morning and evening.

CASE STUDY - QUESTIONS

1. Which of the following actions best demonstrates intra-professional collaboration?
 - a) Discuss adding pantoprazole 40 mg PO daily with the nurse practitioner
 - b) Refer JS to the dietician for counselling on his dietary habits
 - c) Consult an experienced pharmacist to discuss adding gastroprotection or finding an alternative medication to naproxen
 - d) Contacting the physiotherapist to discuss his progress in physical therapy
2. Which of the following best reflects the pharmacist's role in interprofessional care?
 - a) Discontinuing naproxen and initiating intra-articular triamcinolone acetonide
 - b) Initiate pantoprazole 40 mg PO daily
 - c) Communicating drug therapy problems to JS's family health team
 - d) Ordering further laboratory tests
3. Which collaborative goal most directly supports patient safety in JS's case?
 - a) Recommending a single-pill combination of empagliflozin/metformin
 - b) Asking the nurse practitioner to perform weekly blood pressure measurements
 - c) Scheduling a medication review in 1 month
 - d) Recommending stopping naproxen and starting acetaminophen 650 mg Q4H PRN for knee pain
4. Which of the following actions does NOT represent interprofessional collaboration?
 - a) Asking your pharmacy technician to demonstrate correct blood pressure measurement technique to JS
 - b) Speaking with JS's family physician about his naproxen therapy
 - c) Discussing lifestyle interventions with the dietitian
 - d) Recommending combined therapy of his diabetes medications with the nurse practitioner

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CHANGE LOG

July 2025

- Formatting and grammar changes throughout
- Slide 3: Updated legend
- Slide 20: Updated hyperlink
- Slide 48: Takeaway added
- Slides 49, 50: Case study and questions added

November 2025

- No content changes made

March 2026

- Slide 2: Updated NAPRA competencies
- Slide 7: Removed redundancies and made a note for expansion on next slide
- Slide 8: Added clinics and POCT
- Slide 41: Updated names for delegated acts in Canadian provinces